

Women's Health Requirements Discovery Pack

Workflow And Operations

Please confirm how the service should work in practice, especially where the process differs from the written guideline.

Prepared for requirement discussion.

Topic	Our current understanding	Please confirm or amend	Needed in phase 1? (Yes / No / Later)	Owner	Notes
When a new referral arrives	A new referral becomes an item for review and grading.		Yes		
Referral intake	The service may need to accept a referral pack with letters and supporting documents.		Yes		
One-page summary	The summary should be reviewed before grading so the grader does not need to open multiple documents unnecessarily.		Yes		
Access to source documents (letters, labs, imaging)	The grader may still need access to source material behind the summary in some cases.		Maybe		
Suggested grading outcome	The system may provide a suggested outcome once enough information is available.		Yes		
Clinician confirmation	A clinician should be able to accept or change the outcome before it is final.		Yes		
SMO review	Some cases may need SMO-only or senior review handling.		Yes		
Coordinator workflow	A coordinator may need a queue and booking handoff after grading.		Maybe		
How a graded referral reaches the booking team today	There is likely a handoff from grader to booking or scheduling staff.		Yes		
How the grader communicates back to the GP or referrer today	Some outcomes likely need to go back to the GP or referrer, especially returns or rejects.		Yes		
Return to GP	Some referrals may need to be returned to GP or declined rather than booked.		Yes		

Topic	Our current understanding	Please confirm or amend	Needed in phase 1? (Yes / No / Later)	Owner	Notes
Needs more information	Some referrals may need more information before a safe decision can be made.		Yes		
Printable output	A summary and outcome may need to be printed or exported.		Maybe		
Internal notes	Some notes may need to remain internal only.		Yes		
Referrer-facing notes	Some notes or outcomes may need to be sent back to the GP or referrer.		Maybe		
How you track where each referral is in the process	The service likely uses real operational statuses or queues that should be reflected in the future workflow.		Yes		
How urgency and timeframes should be shown	Urgency may need to be shown as a timeframe, category, clinic type, or a combination of these.		Yes		
How FSA or wait-time targets are currently tracked	The service may already track referral timeframes against targets today.		Maybe		
What happens when a referral needs to be escalated after initial grading	The guideline suggests some referrals may need re-grading or escalation after new information arrives.		Yes		
How HSC patients are currently tracked	There may be a specific process for higher-risk or HSC patients, including nurse coordinator involvement.		Yes		
Who currently grades referrals and how many people are involved	The workflow may differ depending on whether grading is done by one person or multiple roles.		Yes		
Reports	The service may need activity, backlog, turnaround, or quality reporting.		No		
Pilot scope	The first pilot may need to cover only one service or a subset of categories.		Yes		